ICU Admission Note

SYNTHETIC DOCUMENT - generated for software testing, not a real patient record

Case ID	SYN-0019	**Age / Sex**	61 / M
Setting	Neuro-ICU	**Primary dx**	Basilar artery occlusion, brainstem stroke

Chief complaint / reason for admission

Sudden loss of consciousness with quadriparesis.

History of present illness

Three days of intermittent vertigo, diplopia and dysarthria (stuttering course). This morning found unresponsive with irregular breathing. Intubated by EMS. Estimated 5 hours from deterioration to arrival.

Past history / medications

Hypertension, hyperlipidemia, type 2 diabetes, current smoker. Not on antithrombotics.

Examination

Intubated. Pupils pinpoint (2 mm), poorly reactive. Absent oculocephalic reflexes, bilateral facial weakness. Quadriparesis with extensor response to pain in the arms and no response in the legs. Plantars extensor.

Glasgow Coma Scale

No eye opening. Verbal not testable (intubated, scored 1T). Motor: extension (decerebrate) response to pain in the arms.

GCS on ICU admission	Score	Response
Eye opening (E)	1	None
Verbal (V)	1T	Intubated (T)
Motor (M)	2	Extension (decerebrate)
TOTAL GCS		**4T**

Investigations

CTA: mid-basilar artery occlusion. Non-contrast CT: no hemorrhage, early hypodensity of the pons. MRI DWI: pontine infarct, pc-ASPECTS 7. Glucose 11.8 mmol/L.

Assessment

Acute basilar artery occlusion with brainstem stroke and severe depression of consciousness. GCS 4T. Potentially salvageable tissue in the cerebellum and mesencephalon.

Plan / disposition

Emergent endovascular thrombectomy at 6 h from onset of coma. Post-procedure BP and glucose management, antiplatelet after follow-up CT. Discussion with family about prognosis, risk of locked-in state, and goals of care.